Thyrocare

Tests you can trust

Name : Srinivasan M K(37Y/M)

Date : 15 Feb 2025

Test Asked : Fk Health Management Test

Report Status: Complete Report

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Thyrocare
Seshans Complex, No.140,
Anna Salai, Saidapet,
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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation#

NAME : SRINIVASAN M K(37Y/M)
REF. BY : SELF
TEST ASKED : FK HEALTH MANAGEMENT TEST

HOME COLLECTION :
Plot No 78A, Second Main Road, South Lakshmi Nagar, Mudichur, Chennai, Chengelpattu, Tamil nadu Second Main Road

Report Availability Summary

Note: Please refer to the table below for status of your tests.

13 Ready | **0** Ready with Cancellation | **1** Processing | **0** Cancelled in Lab

TEST DETAILS	REPORT STATUS
FK HEALTH MANAGEMENT TEST	Partially Ready
LIPID PROFILE	Ready
T3-T4-USTSH	Ready
FASTING BLOOD SUGAR(GLUCOSE)	Ready
KIDPRO	Ready
VITAMIN B-12	Ready
IRON	Ready
LIVER FUNCTION TESTS	Ready
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	Ready
Lipoprotein (a) [Lp(a)]	Ready
APOLIPROTEIN RATIO	Ready
25-OH VITAMIN D (TOTAL)	Ready
ERYTHROCYTE SEDIMENTATION RATE (ESR)	Ready
HBA PROFILE	Ready
HEMOGRAM - 6 PART (DIFF)	Processing

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Summary Report

Tests outside reference range

TEST NAME	OBSERVED VALUE	UNITS	Bio. Ref. Interval.
ABSOLUTE COUNTS			
EOSINOPHILS - ABSOLUTE COUNT	1.44	X 10³ / µL	0.02 - 0.5
CARDIAC RISK MARKERS			
Lipoprotein (a) [Lp(a)]	37.71	mg/dL	< 30
DIFFERENTIAL COUNTS			
EOSINOPHILS	16	%	1-6
LIPID			
HDL / LDL RATIO	0.24	Ratio	> 0.40
HDL CHOLESTEROL - DIRECT	38	mg/dL	40-60
LDL / HDL RATIO	4.1	Ratio	1.5-3.5
LDL CHOLESTEROL - DIRECT	154	mg/dL	< 100
NON-HDL CHOLESTEROL	175.9	mg/dL	< 160
TC/ HDL CHOLESTEROL RATIO	5.7	Ratio	3 - 5
TOTAL CHOLESTEROL	214	mg/dL	< 200
TRIG / HDL RATIO	4.52	Ratio	< 3.12
TRIGLYCERIDES	171	mg/dL	< 150
LIVER			
ALANINE TRANSAMINASE (SGPT)	67.8	U/L	< 45
OTHER COUNTS			
ERYTHROCYTE SEDIMENTATION RATE (ESR)	25	mm / hr	0 - 15
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	35.2	pq	27.0-32.0
MEAN CORPUSCULAR VOLUME(MCV)	105.2	fL	83.0-101.0
PLATELET DISTRIBUTION WIDTH(PDW)	8.5	fL	9.6-15.2
PLATELET TO LARGE CELL RATIO(PLCR)	12	%	19.7-42.4
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	52.2	fL	39-46
TOTAL RBC	4.4	X 10^6/µL	4.5-5.5
RENAL			
BLOOD UREA NITROGEN (BUN)	7.33	mg/dL	7.94 - 20.07
UREA (CALCULATED)	15.69	mg/dL	Adult : 17-43
URIC ACID	3.3	mg/dL	4.2 - 7.3
VITAMIN			
25-OH VITAMIN D (TOTAL)	19.83	ng/mL	30-100
VITAMIN B-12	188	pg/mL	211-911

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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TEST ASKED : FK HEALTH MANAGEMENT TEST

HOME COLLECTION :
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TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c - (HPLC)			
	H.P.L.C	4.9	%

Bio. Ref. Interval. :

Bio. Ref. Interval.: As per ADA Guidelines	Guidance For Known Diabetics
Below 5.7% : Normal	Below 6.5% : Good Control
5.7% - 6.4% : Prediabetic	6.5% - 7% : Fair Control
>=6.5% : Diabetic	7.0% - 8% : Unsatisfactory Control
	>8% : Poor Control

Method : Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG)	CALCULATED	95	mg/dL

Bio. Ref. Interval. :

90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control

Method : Derived from HBA1c values

Please correlate with clinical conditions.

Sample Collected on (SCT) : 15 Feb 2025 08:50
Sample Received on (SRT) : 15 Feb 2025 12:04
Report Released on (RRT) : 15 Feb 2025 16:04
Sample Type : EDTA Whole Blood
Labcode : 1502040635/DI191
Barcode : DA168205

Dr D Sanjuna MD (Path)

S. Nalina Priya

Dr Nalina MD (Path)

Scan QR code to verify authenticity of reported results; active for 30 days from release time.

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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	15.5	g/dL	13.0-17.0
Hematocrit (PCV)	CPH Detection	46.3	%	40.0-50.0
Total RBC	**HF & EI**	**4.4**	**X 10^6/µL**	**4.5-5.5**
Mean Corpuscular Volume (MCV)	**Calculated**	**105.2**	**fL**	**83.0-101.0**
Mean Corpuscular Hemoglobin (MCH)	**Calculated**	**35.2**	**pq**	**27.0-32.0**
Mean Corp.Hemo. Conc (MCHC)	Calculated	33.5	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	**Calculated**	**52.2**	**fL**	**39-46**
Red Cell Distribution Width (RDW - CV)	Calculated	13.1	%	11.6-14
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	9.02	X 10^3 / µL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	51.1	%	40-80
Lymphocytes Percentage	Flow Cytometry	27.5	%	20-40
Monocytes Percentage	Flow Cytometry	4.5	%	2-10
Eosinophils Percentage	**Flow Cytometry**	**16**	**%**	**1-6**
Basophils Percentage	Flow Cytometry	0.7	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.2	%	0-0.5
Nucleated Red Blood Cells %	Flow Cytometry	0.1	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	4.61	X 10^3 / µL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	2.48	X 10^3 / µL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.41	X 10^3 / µL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.06	X 10^3 / µL	0.02 - 0.1
Eosinophils - Absolute Count	**Calculated**	**1.44**	**X 10^3 / µL**	**0.02 - 0.5**
Immature Granulocytes (IG)	Calculated	0.02	X 10^3 / µL	0-0.3
Nucleated Red Blood Cells	Calculated	0.009	X 10^3 / µL	0.0-0.5
PLATELET COUNT	HF & EI	344	X 10^3 / µL	150-410
Mean Platelet Volume (MPV)	Calculated	8.4	fL	6.5-12
Platelet Distribution Width (PDW)	**Calculated**	**8.5**	**fL**	**9.6-15.2**
Platelet to Large Cell Ratio (PLCR)	**Calculated**	**12**	**%**	**19.7-42.4**
Plateletcrit (PCT)	Calculated	0.29	%	0.19-0.39
Others				
ERYTHROCYTE SEDIMENTATION RATE (ESR)	**MODIFIED WESTERGREN**	**25**	**mm / hr**	**0 - 15**

Clinical history is asked for all the relevant abnormalities detected and in absence / failure of receiving of clinical history, results are rechecked twice and released. Advised clinical correlation.
Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)
(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height**)**

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Sample Received on (SRT) : 15 Feb 2025 12:04
Report Released on (RRT) : 15 Feb 2025 16:04
Sample Type : EDTA Whole Blood
Labcode : 1502040635/DI191
Barcode : DA168205

Dr D Sanjuna MD (Path)

S. Nalina Priya

Dr Nalina MD (Path)

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TEST NAME	TECHNOLOGY	VALUE	UNITS
FASTING BLOOD SUGAR(GLUCOSE)	PHOTOMETRY	86.1	mg/dL

Bio. Ref. Interval. :-

As per ADA Guideline: Fasting Plasma Glucose (FPG)	
Normal	70 to 100 mg/dl
Prediabetes	100 mg/dl to 125 mg/dl
Diabetes	126 mg/dl or higher

Note :
The assay could be affected mildly and may result in anomalous values if serum samples have heterophilic antibodies, hemolyzed , icteric or lipemic. The concentration of Glucose in a given specimen may vary due to differences in assay methods, calibration and reagent specificity. For diagnostic purposes results should always be assessed in conjunction with patients medical history, clinical findings and other findings.

Please correlate with clinical conditions.
Method:- GOD-PAP METHOD

Sample Collected on (SCT) : 15 Feb 2025 08:50
Sample Received on (SRT) : 15 Feb 2025 12:04
Report Released on (RRT) : 15 Feb 2025 13:31
Sample Type : FLUORIDE PLASMA
Labcode : 1502070517/DI191
Barcode : DL829150

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TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	**C.L.I.A**	**19.83**	**ng/mL**

Bio. Ref. Interval. :-

DEFICIENCY : <20 ng/ml || INSUFFICIENCY : 20-<30 ng/ml
SUFFICIENCY : 30-100 ng/ml || TOXICITY : >100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health. Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome. Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml.

Kit Validation Reference: Holick MF. Vitamin D Deficiency. N Engl J Med. 2007;357:266–81.

Please correlate with clinical conditions.
Method:- Fully Automated Chemi Luminescent Immuno Assay

Sample Collected on (SCT) : 15 Feb 2025 08:50
Sample Received on (SRT) : 15 Feb 2025 12:04
Report Released on (RRT) : 15 Feb 2025 17:47
Sample Type : SERUM
Labcode : 1502070492/DI191
Barcode : CZ463251

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TEST NAME	TECHNOLOGY	VALUE	UNITS
APOLIPOPROTEIN - A1 (APO-A1)	IMMUNOTURBIDIMETRY	124	mg/dL

Bio. Ref. Interval. :
Male : 86 - 152
Female : 94 - 162
Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APOLIPOPROTEIN - B (APO-B)	IMMUNOTURBIDIMETRY	118	mg/dL

Bio. Ref. Interval. :
Male : 56 - 145
Female : 53 - 138
Method : FULLY AUTOMATED RATE IMMUNOTURBIDIMETRY – BECKMAN COULTER

APO B / APO A1 RATIO (APO B/A1)	CALCULATED	0.9	Ratio

Bio. Ref. Interval. :

Male : 0.40 - 1.26
Female : 0.38 - 1.14

Clinical Significance :

- Apolipoprotein B is a more potent and independent predictor of Coronary artery disease (CAD) than LDL Cholesterol.
- Apolipoprotein A1 is one of the apoproteins of HDL and is inversely related to risk of CAD.
- The Apolipoprotein studies help in monitoring risk of restenosis in patients with myocardial infarction, Coronary bypass surgery etc.
- An increased ratio of Apo B to A1 beyond the defined normal range is indicative of CAD risk.
- All results have to be interpreted in Conjunction with clinical history and other findings.

Method : Derived from serum Apo A1 and Apo B values

Please correlate with clinical conditions.

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Sample Type : SERUM
Labcode : 1502070492/DI191
Barcode : CZ463251

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	IMMUNOTURBIDIMETRY	1.98	mg/L

Bio. Ref. Interval. :-

< 1.00 - Low Risk
1.00 - 3.00 - Average Risk
>3.00 - 10.00 - High Risk
> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection , active arthritis or concurrent illness.

Clinical significance:
High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:
1.Clinical management of laboratory date in medical practice 2003-3004, 207(2003).
2.Tietz : Textbook of Clinical Chemistry and Molecular diagnostics :Second edition :Chapter 47:Page no.1507- 1508.

Please correlate with clinical conditions.
Method:- FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER

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Sample Type : SERUM
Labcode : 1502070492/DI191
Barcode : CZ463251

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TEST NAME	TECHNOLOGY	VALUE	UNITS
VITAMIN B-12	**C.L.I.A**	**188**	**pg/mL**

Bio. Ref. Interval. :-

Normal : 211 - 911 pg/ml

Clinical significance :
Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):5.0%, Inter assay (%CV):9.2 %;Sensitivity:45 pg/ml

Kit Validation reference:
Chen IW, Sperling MI, Heminger LA. Vitamin B12. In: Pesce AJ, Kaplan LA, eds. Methods in Clinical Chemistry. St. Louis: CV Mosby; 1987:569–73.

Please correlate with clinical conditions.
Method:- COMPETITIVE CHEMI LUMINESCENT IMMUNO ASSAY

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TEST NAME	TECHNOLOGY	VALUE	UNITS
Lipoprotein (a) [Lp(a)]	**IMMUNOTURBIDIMETRY**	**37.71**	**mg/dL**

Bio. Ref. Interval. :-

Adults : < 30.0 mg/dl

Clinical Significance:
Determination of LPA may be useful to guide management of individuals with a family history of CHD or with existing disease. The levels of LPA in the blood depends on genetic factors; The range of variation in a population is relatively large and hence for diagnostic purpose, results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

Specifications:
Precision %CV :- Intra assay %CV- 4.55% , Inter assay %CV-0.86 %

Kit Validation Reference:
Tietz NW,Clinical Guide to Laboratory Tests Philadelphia WB. Saunders 1995 : 442-444

Please correlate with clinical conditions.
Method:- LATEX ENHANCED IMMUNOTURBIDIMETRY

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	**PHOTOMETRY**	**214**	**mg/dL**	**< 200**
HDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**38**	**mg/dL**	**40-60**
LDL CHOLESTEROL - DIRECT	**PHOTOMETRY**	**154**	**mg/dL**	**< 100**
TRIGLYCERIDES	**PHOTOMETRY**	**171**	**mg/dL**	**< 150**
TC/ HDL CHOLESTEROL RATIO	**CALCULATED**	**5.7**	**Ratio**	**3 - 5**
TRIG / HDL RATIO	**CALCULATED**	**4.52**	**Ratio**	**< 3.12**
LDL / HDL RATIO	**CALCULATED**	**4.1**	**Ratio**	**1.5-3.5**
HDL / LDL RATIO	**CALCULATED**	**0.24**	**Ratio**	**> 0.40**
NON-HDL CHOLESTEROL	**CALCULATED**	**175.9**	**mg/dL**	**< 160**
VLDL CHOLESTEROL	CALCULATED	34.16	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
LDL - Direct Measure
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
TRI/H - Derived from TRIG and HDL Values
LDL/ - Derived from serum HDL and LDL Values
HD/LD - Derived from HDL and LDL values.
NHDL - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	90.05	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.56	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.1	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.46	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	44.1	U/L	< 55
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	33.9	U/L	< 35
ALANINE TRANSAMINASE (SGPT)	**PHOTOMETRY**	**67.8**	**U/L**	**< 45**
SGOT / SGPT RATIO	CALCULATED	0.5	Ratio	< 2
PROTEIN - TOTAL	PHOTOMETRY	7.95	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.64	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.31	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.4	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
OT/PT - Derived from SGOT and SGPT values.
PROT - Biuret Method
SALB - Albumin Bcg[1]method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

Sample Collected on (SCT) : 15 Feb 2025 08:50
Sample Received on (SRT) : 15 Feb 2025 12:04
Report Released on (RRT) : 15 Feb 2025 17:47
Sample Type : SERUM
Labcode : 1502070492/DI191
Barcode : CZ463251

Dr D Sanjuna MD (Path)

S. Nalina Priya
Dr Nalina MD (Path)

PROCESSED AT :
Thyrocare
Seshans Complex, No.140,
Anna Salai, Saidapet,
Chennai - 600 015

Thyrocare
Tests you can trust

Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703 9870666333 wellness@thyrocare.com

First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation#

NAME : SRINIVASAN M K(37Y/M)
REF. BY : SELF
TEST ASKED : FK HEALTH MANAGEMENT TEST

HOME COLLECTION :
Plot No 78A, Second Main Road, South Lakshmi Nagar, Mudichur, Chennai, Chengelpattu, Tamil nadu Second Main Road

TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON	PHOTOMETRY	110	µg/dL

Bio. Ref. Interval. :
Male : 65 - 175
Female : 50 - 170
Method : Ferrozine method without deproteinization

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	**PHOTOMETRY**	**7.33**	**mg/dL**	**7.94 - 20.07**
CREATININE - SERUM	PHOTOMETRY	0.79	mg/dL	0.72-1.18
BUN / SR.CREATININE RATIO	CALCULATED	9.28	Ratio	9:1-23:1
UREA (CALCULATED)	**CALCULATED**	**15.69**	**mg/dL**	**Adult : 17-43**
UREA / SR.CREATININE RATIO	CALCULATED	19.86	Ratio	< 52
CALCIUM	PHOTOMETRY	9.92	mg/dL	8.8-10.6
URIC ACID	**PHOTOMETRY**	**3.3**	**mg/dL**	**4.2 - 7.3**

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
SCRE - Creatinine Enzymatic Method
B/CR - Derived from serum Bun and Creatinine values
UREAC - Derived from BUN Value.
UR/CR - Derived from UREA and Sr.Creatinine values.
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	C.M.I.A	120	ng/dL	58-159
TOTAL THYROXINE (T4)	C.M.I.A	9.06	µg/dL	4.87-11.72
TSH - ULTRASENSITIVE	C.M.I.A	2.46	µIU/mL	0.35-4.94

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3,T4,USTSH - Fully Automated Chemi Luminescent Microparticle Immunoassay

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

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TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	117	mL/min/1.73 m2

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.
Method:- 2021 CKD EPI Creatinine Equation

~~ End of report ~~

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Scan QR code to verify authenticity of reported results; active for 30 days from release time.

CONDITIONS OF REPORTING

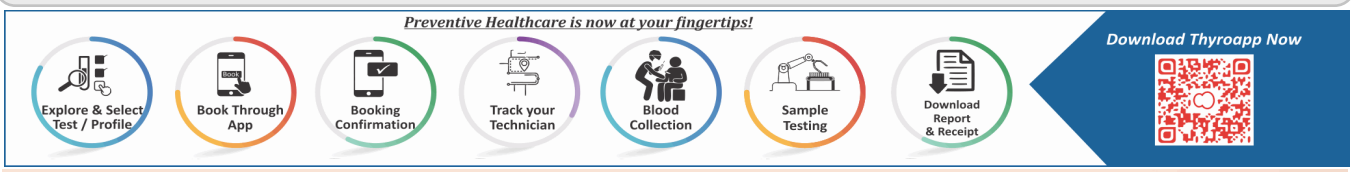
- The reported results are for information and interpretation of the referring doctor only.
- It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- This report is not valid for medico-legal purpose.
- Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- Thyrocare Discovery video link :- https://youtu.be/nbdYeRgYyQc

EXPLANATIONS

- Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
- **Barcode** - This is the specimen identity number and it states that the results are for the specimen bearing the barcode (irrespective of the name).
- **SCP** - Specimen Collection Point - This is the location where the blood or specimen was collected as declared by the client.
- **SCT** - Specimen Collection Time - The time when specimen was collected as declared by the client.
- **SRT** - Specimen Receiving Time - This time when the specimen reached our laboratory.
- **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- Values out of reference range requires reconfirmation before starting any medical treatment.
- Retesting is needed if you suspect any quality shortcomings.
- Testing or retesting should be done in accredited laboratories.
- For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**



*+T&C Apply, #As on 5th December 2024, *As per a survey on doctors' perception of laboratory diagnostics (IJARIIT,2023)*